institutions were used to justify and facilitate discrimination, imperialism and even genocide. These crimes and errors are not blamed on a few misguided scholars. They are seen as an institutional failure of entire academic disciplines.[107]

The willingness to admit major institutional errors contributes to the relatively fast pace at which science is developing. When the available evidence justifies it, dominant theories are often discarded within a few generations, to be replaced by new theories. What students of biology, anthropology and history learn at university in the early twenty-first century is very different from what they learned there a century previously.

Psychiatry offers numerous similar examples for strong self-correcting mechanisms. On the shelf of most psychiatrists you can find the *DSM* – the *Diagnostic and Statistical Manual of Mental Disorders*. It is occasionally nicknamed the psychiatrists' bible. But there is a crucial difference between the *DSM* and the Bible. First published in 1952, the *DSM* is revised every decade or two, with the fifth edition appearing in 2013. Over the years, many disorders have been redefined, new ones have been added, while others have been deleted. Homosexuality, for example, was listed in 1952 as a sociopathic personality disturbance, but removed from the *DSM* in 1974. It took just twenty-two years to correct this error in the *DSM*. That's not a holy book. That's a scientific text.

Today the discipline of psychiatry doesn't try to reinterpret the 1952 definition of homosexuality in a more benign spirit. Rather, it views the 1952 definition as a downright error. More important, the error is not attributed to the shortcomings of a few homophobic professors. Rather, it is acknowledged to be the result of deep institutional biases in the discipline of psychiatry.[108] Confessing the past institutional errors of their discipline makes psychiatrists today more careful not to commit new such errors, as evidenced in the heated debate regarding transgender people and people on the autistic spectrum. Of course, no matter how careful they are, psychiatrists are still likely to make institutional mistakes. But they are also likely to acknowledge and correct them.[109]